TASK 5: INDIVIDUAL CASE SAFETY REPORT (ICSR)

Pharmacovigilance Internship • Fictional Practice Assessment

Scope note: The screenshot shows the task requirements but not the underlying case study. This is a clearly labelled fictional practice example to demonstrate the workflow. Replace illustrative details with the actual tutorial case before submitting if case-specific answers are required.

Practice Case Summary

Fictional scenario: An adult takes amoxicillin for a suspected bacterial infection and develops an itchy skin rash the following day. The medicine is stopped after clinical advice and the rash improves. No hospitalization, life-threatening event, disability, or other serious outcome is described in this illustrative scenario.

Field	Illustrative entry
Case ID	PRACTICE-ICSR-01 (fictional; not a real report)
Suspected medicine	Amoxicillin; illustrative example
Adverse reaction	Itchy skin rash; illustrative example
Onset	Following day after medicine started; exact dates not supplied
Action taken	Medicine stopped after clinical advice in this fictional scenario
Outcome	Improved in this fictional scenario

01. ADR Identification

Suspected ADR: Itchy skin rash temporally associated with amoxicillin in the fictional case. The reaction is identified for assessment; this alone does not prove that the medicine caused it.

Missing information for a real case: required patient details, dose and route, indication, exact start/stop dates, onset and resolution dates, relevant history, concomitant medicines, treatment, and reporter details. Do not invent missing facts or unnecessary patient identifiers.

02. ICSR Reporting — Practice Form

ICSR field	Illustrative entry / status
Patient information	Adult; age, sex and other required details not provided — leave unknown rather than inventing.
Suspected medicine	Amoxicillin; dose, route, frequency, indication, batch number and treatment dates are unavailable in the screenshot.
Reaction details	Itchy skin rash; onset described only as the following day in this fictional example. Exact dates and clinical description require confirmation.
Concomitant medicines	Unknown — verify from the actual case study.
Relevant history / tests	Not provided — do not assume absent.
Action taken / treatment	Medicine stopped after clinical advice in this fictional scenario; other treatment details unknown.
Outcome	Improved in this fictional scenario; exact resolution date not provided.
Reporter / report date	Not supplied; complete only with actual authorized reporter details and date.

03. Causality Assessment

Preliminary assessment: Timing and improvement after stopping the medicine may support a possible association, but the fictional details are insufficient for a definitive causality category. Consider timing, alternative causes, concomitant medicines, dechallenge/rechallenge information where available, and the method required by the training task.

Practice conclusion: Causality is not definitively established from these limited illustrative facts. Do not label it certain or probable without sufficient case evidence and the required assessment criteria.

04. Severity & Seriousness Classification

Assessment	Practice classification	Basis / limitation
Symptom severity	Cannot be precisely graded	The scenario says “itchy rash” but provides no scale or detailed clinical findings.
Regulatory seriousness	Provisionally non-serious in this example	No serious outcome is described; confirm all criteria using the actual case.
Expectedness	Undetermined	Compare the reaction with applicable approved product information / reference safety information.
Clinical impact	Limited in this fictional example	Rash reportedly improves; no admission or major intervention is described.

Final Practice Summary

This fictional example demonstrates all four task components: ADR identification, ICSR reporting, causality assessment, and severity/seriousness classification. It is not an actual patient report and must not be presented as one. The actual tutorial case was not included in the screenshot, so case-specific fields and final classifications must be verified against that source.

Reminder: Severity describes symptom intensity; seriousness is based on regulatory outcome criteria. Expectedness requires reference safety information. Do not invent missing patient or reporter data.